03. ORIGIN OF THE EYE

DURATION : 07:12 STUDY SHEET

COURSE SUMMARY

This video explores in depth the origin of the eye, which develops from the diencephalon, from the eighth day of embryonic development. The emphasis is placed on the importance of the amniotic cavity and cerebrospinal fluid (CSF), as well as how these elements influence the formation of the eye and its dynamic balance with the environment. The relationship between the amniotic cavity, the ventricular system, and the eye is highlighted, while also underlining the clinical and genetic implications that can affect the individual's health and manifest through the eye. This understanding can help therapists assess and restore balance in their patients, making the teaching essential for those who practice osteopathy and biodynamic embryology.

ORIGIN OF THE EYE

The eye shows a fascinating **resemblance** to its formation on the eighth day of embryonic development, when a small cavity called the **amniotic cavity** appears. This cavity is fundamental for the development of the eye, which originates from the **brain**, more precisely from the **diencephalon**, an expansion of the third ventricle.

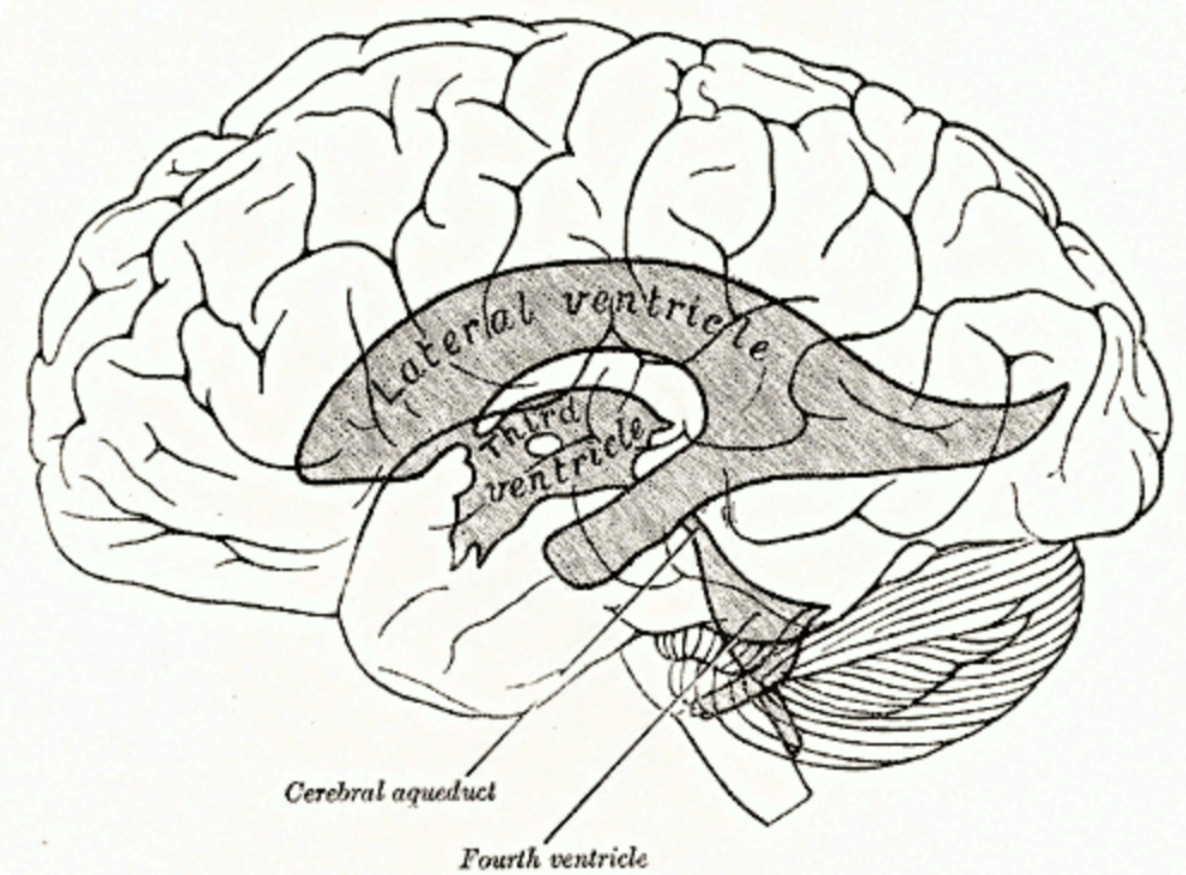

FIG. — Diagram related to the origin of the eye from the diencephalon, in connection with the third ventricle

During this development, the neural system receives **primitive amniotic fluid**, which will form the **cerebrospinal fluid (CSF)**. This initial CSF, derived from amniotic fluid, will be enclosed within the neural tube. Subsequently, this tube opens anteriorly, forming **primitive vesicles**.

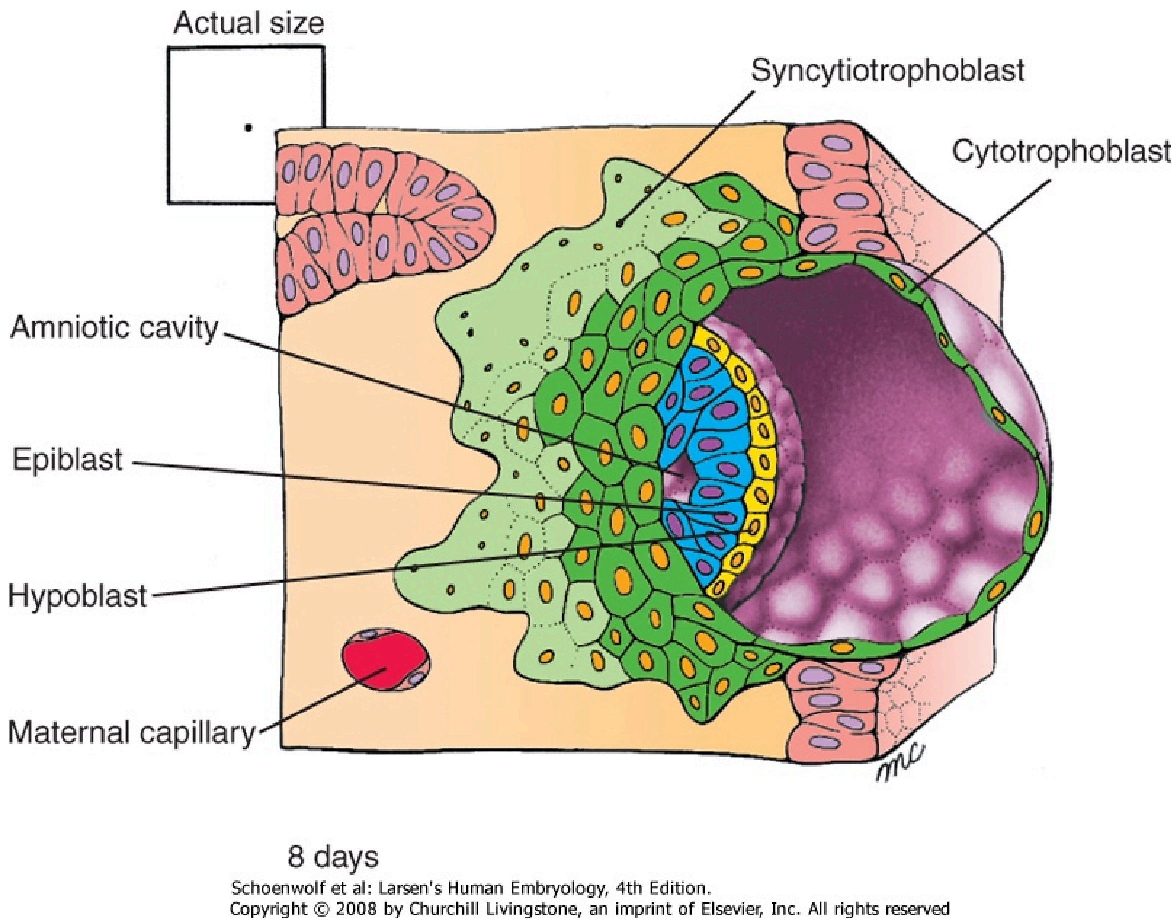

FIG. — Diagram of the appearance of the amniotic cavity, which is mentioned just after this sentence in the summary

The remnant of this amniotic cavity is found in what is called the **bone pocket** or **zone B**, where cerebrospinal fluid is located. It is interesting to note that the fluid inside and outside the embryo is the same. The eye, as a membrane, constantly seeks to balance this zone B.

Zone A is represented by the central axis of the physical body, while the amniotic cavity begins to form on the eighth day, marking the entry into the **mucosa**. At this stage, a central fluid loss creates a small cavity, and the **ectoderm**, called the **epiblast** at this stage, begins to establish itself. This already constitutes the subtle structure that will form the eye and the brain.

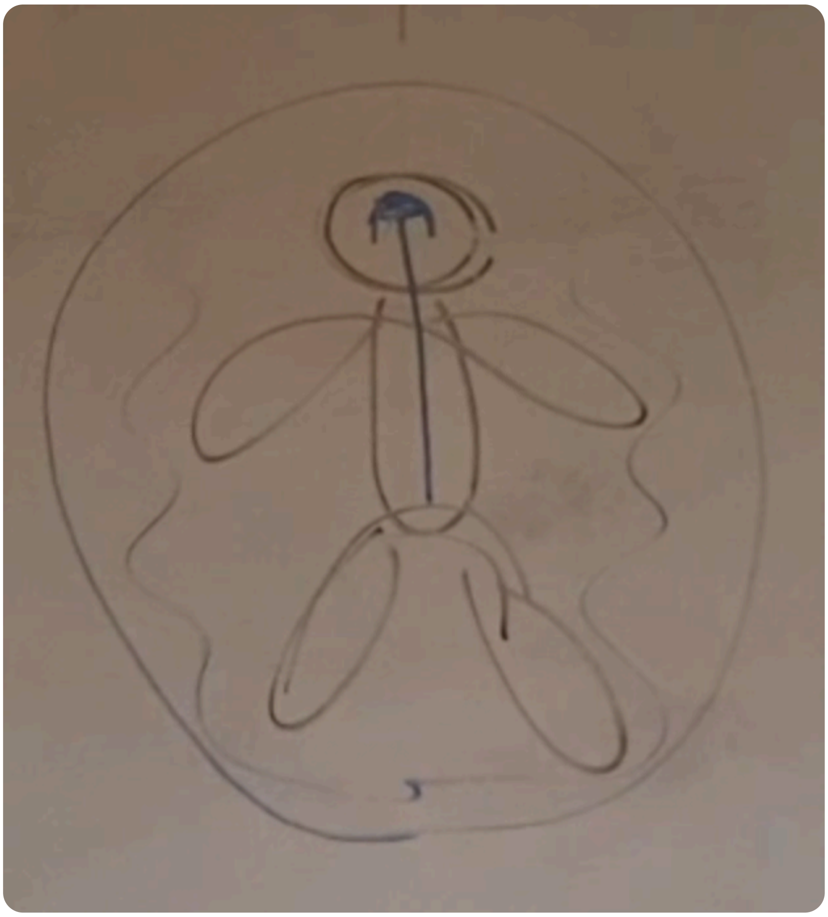

FIG. — Vestige of the amniotic cavity (zone B)

The amniotic cavity, which surrounds the embryo for nine months, then becomes a vaporous space. This space must be in balance with the internal fluid. Imbalances can occur, particularly in the event of accidents, affecting the position of the eye. The eye thus represents a balance between the depth of the fluid of the nervous system (CSF) and the surrounding space.

Observing the position of the eyes can reveal information about a person's balance. Each individual has a slightly different eye, often related to the architecture of their brain. The search for **horizontality** in relation to **verticality** is essential, as is the balance between the digestive system and the peritoneal system.

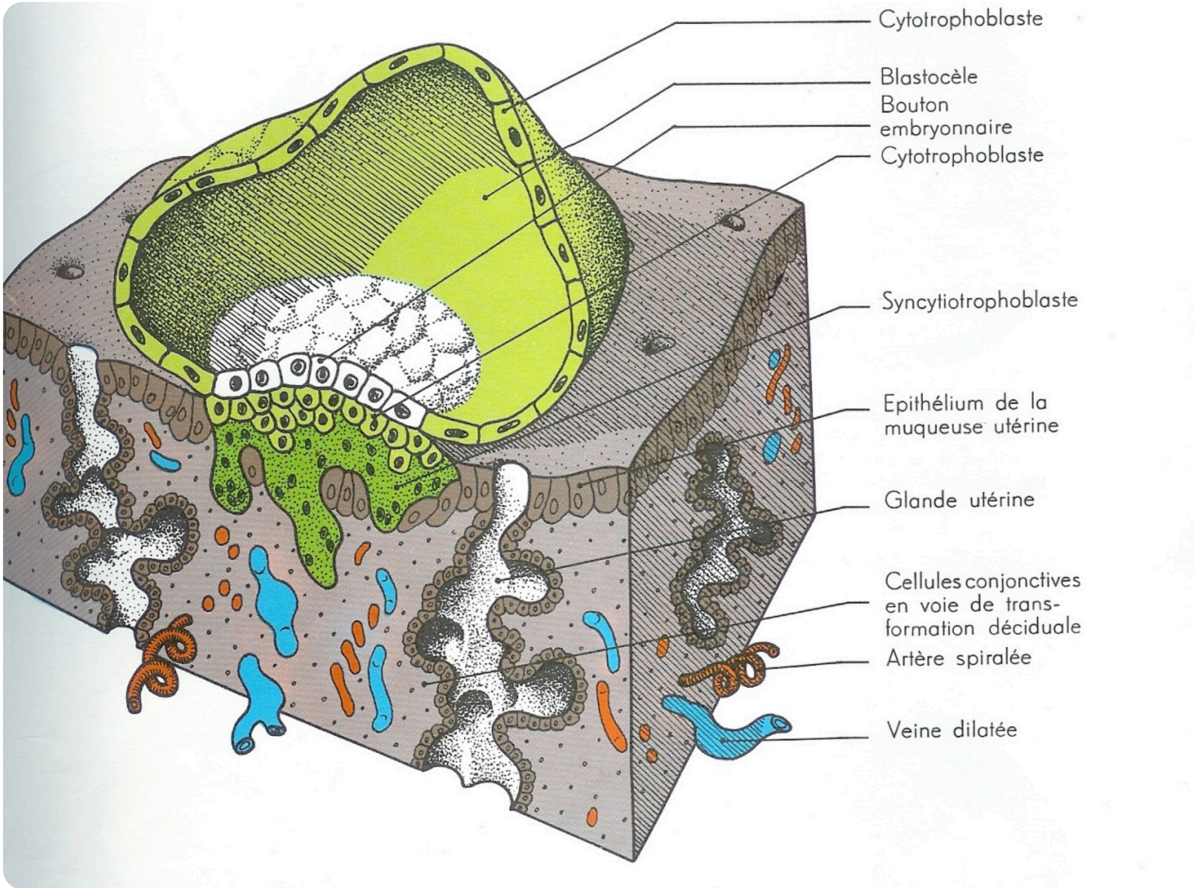

FIG. — Implantation process: cavity formation

It is crucial to observe the eyes after treatment. If the position of the eyes is not stable, it indicates that the balance is not yet restored. The amniotic cavity, although no longer in liquid form, remains present in other states, such as vaporous.

This relationship between the amniotic cavity, the eye, and the ventricular system is fundamental. The eye, as an expansion of the third ventricle, begins to develop before the closure of the **anterior neuropore**. Furthermore, there are interesting genetic implications, with co-localizations of information on impact zones, such as the hepatopancreatic, cardiac, and thyroid systems.

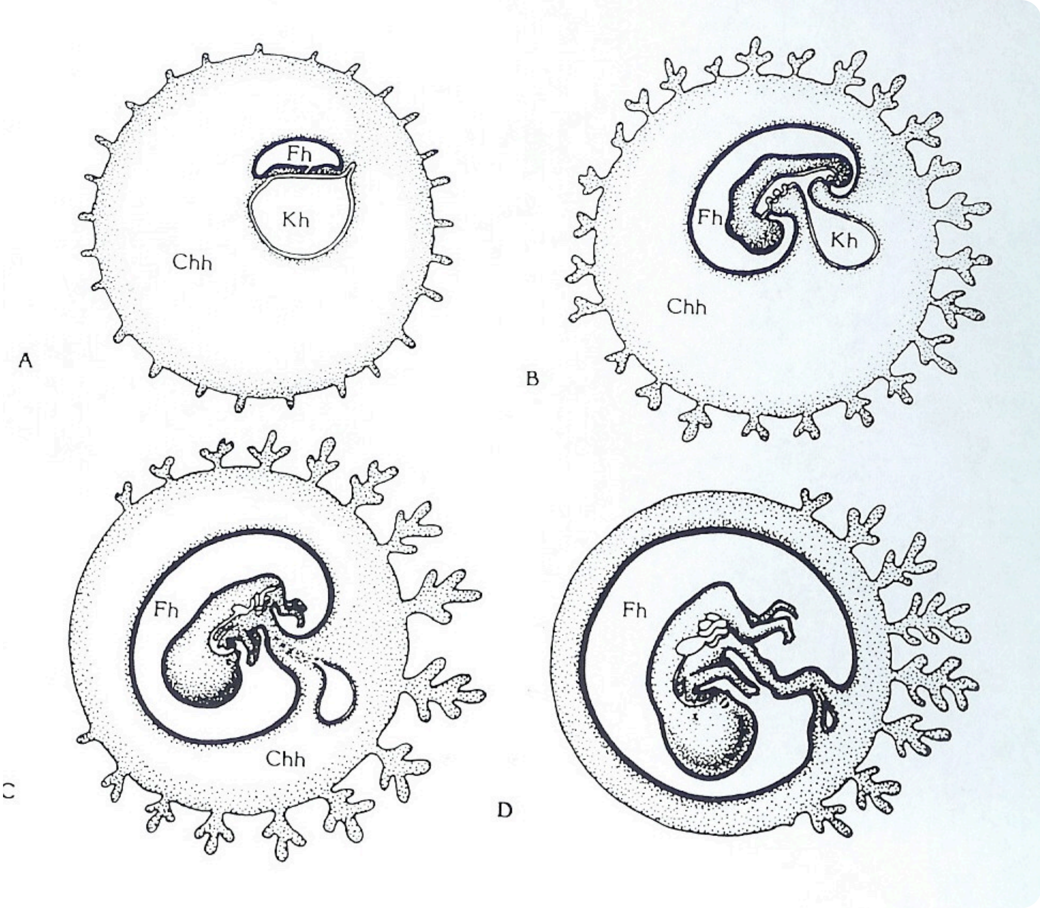

FIG. — Amniotic cavity

Pathologies such as **diabetes**, heart problems, and thyroid issues can manifest through the eye, thus revealing the importance of the **neural crest** in the organization of the midline and laterality.